SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Pembrolizumab 400 mg · every 6 weeks · lung cancer

Lung cancer only: first-line metastatic NSCLC or adjuvant NSCLC after surgery and platinum chemotherapy · NCCP 00558 · version 13 · 42-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
For metastatic lung cancer, the aim is to control cancer and help people live longer. After surgery and platinum chemotherapy, the adjuvant aim is to reduce the chance of cancer returning. Ask which aim applies to you.

Schedule
Pembrolizumab 400 mg is given as a drip over about 30 minutes on Day 1 every 6 weeks (42 days). The hospital visit is longer because of checks and preparation. After complete surgery and platinum chemotherapy, the adjuvant course lasts up to 12 months. For metastatic cancer, treatment continues while it remains helpful and tolerable; the team may consider stopping after 24 months of sustained response without progression, or continuing when appropriate.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Pembrolizumab
IMMUNOTHERAPY

- ☐ Immune inflammation of lungs, bowel, liver, kidneys, hormone glands, skin, heart and nervous system
- ☐ Infusion reactions
- ☐ Some effects need steroids, hospital care or long-term hormone replacement
- ☐ Effects may start after the final dose
- ☐ Heart, muscle and nerve immune effects can occur together and require urgent assessment

Discuss the aim for your exact lung setting, likely benefit, alternatives, important immune risks and how treatment may be paused or stopped. Some side effects need long-term medicines. Ask about fertility, pregnancy, breastfeeding and effects on daily life. For treatment after surgery, distinguish delaying recurrence from an overall-survival benefit, which was not established in the final trial analysis. Discuss the possibility that heart, muscle and nerve inflammation can occur together and may need urgent hospital treatment. A severe immune reaction may mean treatment must remain stopped even after recovery.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00558 v13.

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